جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : حنان حمدي حسن يوسف
المحافظة : الجيزه **الحي / المدينة :** الهرم **العنوان :** الهرم
رقم البطاقة : 26901062300103 **رقم التليفون :** 01114857412 **تاريخ تسجيل الإستمارة :** 18-10-2026
العمر(تاريخ الميلاد) : 06-01-1969 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي المقر بما فيه
التوقيع:(----------) التوقيع:(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- ا.د/ ---------- د/ ---------- د/ ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: حنان حمدي حسن يوسف
National ID: 26901062300103

GUSTAVE ROUSSY

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشان تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : ---:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

Imaging report

كايرو سكان
للأشعة والتحليل
CAIRO SCAN
Radiology & LABS

Pat. Name: Hanan Hamdy Hassan
Date: 01/10/2025

Pat. ID: 3413269

^{18}F FDG WHOLE BODY FOLLOW UP PET/CT

- Whole body 18F-FDG images from skull to the knees with the arms extending above the head were obtained approximately 1 hour after tracer injection.
- CT was acquired for the purpose of anatomic localization and attenuation correction
- Data was processed and displayed according to departmental protocol.

Review of PET. CT and fused imaaes

Compared to the previous PET/CT scan dated 21.7.2025 , the current study revealed:

BRAIN

- No evidence of intra-cerebral or extra-axial fresh blood density.
- Normal size and configuration of the ventricular system and subarachnoid basal cisterns as well as the cortical sulci on both sides.
- No definite cerebral areas of abnormal attenuation value or pathological enhancement.
- No shift of midline structures.
- Normal posterior fossa.
- Clear scanned para-nasal sinuses.

NECK:

- Normal CT appearance of valeculae, pyriform fossae and aryepigolottic folds.
- Patent nasopharyngeal and oro-pharyngeal air column.
- Bilateral deep cervical, subcentemetric benign looking lymph nodes with no pathological tracer uptake , likely non specific.
- Normal CT appearance of the parotid and submandibular salivary glands.
- Normal appearance of the thyroid gland.
- Normal appearance of the carotid sheath and its contents

Imaging report

كايرو سكان للأشعة والتحليل
CAIRO SCAN
Radiology & LABS

Pat. Name: Hanan Hamdy Hassan
Date: 01/10/2025

Pat. ID: 3413269

CHEST:

- Status post operative with the right breast showing parenchymal soft tissue distortion thickened skin and retracted nipple and deep surgical sutures with no pathological tracer uptake likely representing post operative sequelae. NO FDG avid recurrent neoplastic lesions detected
- Clear both lung fields with no evidence of metabolically active pulmonary nodules, masses, infiltrations or bronchiectatic changes.
- No pleural collection.
- stable minimal pericardial effusion
- No metabolically active grossly enlarged hilar, mediastinal or axillary lymph nodes.
- Normal CT appearance of the heart, aorta and its great branches.
- stable hiatus hernia

ABDOMEN AND PELVIS:

- Enlarged liver showing smooth regular outline with homogenous low parenchymal texture. No evidence of definite metabolically active enhanced focal hepatic masses or bile duct dilatation.
- Patent homogenously enhancing portal vein and its main branches.
- Normal CT appearance of the spleen, pancreas, suprarenal glands, kidneys, aorta, IVC and pelvic organs.
- No metabolically active grossly enlarged abdominal or pelvic lymph nodes.
- No ascites.

MUSCULO-SKELETAL

- **Stationary course of the heterogeneous low grade metabolically active mixed sclerotic and lytic skeletal lesions involving multiple cervico-dorso-lumbar vertebrae , multiple ribs bilaterally, multiple pelvic bones including the sacrum, sternum, both scapulae, both clavicles, both humeri and femora including the right sided osteolytic lesion centered upon the right iliac bone and right inferior pubic ramus with pathological fractures and reduced height/partial structural**

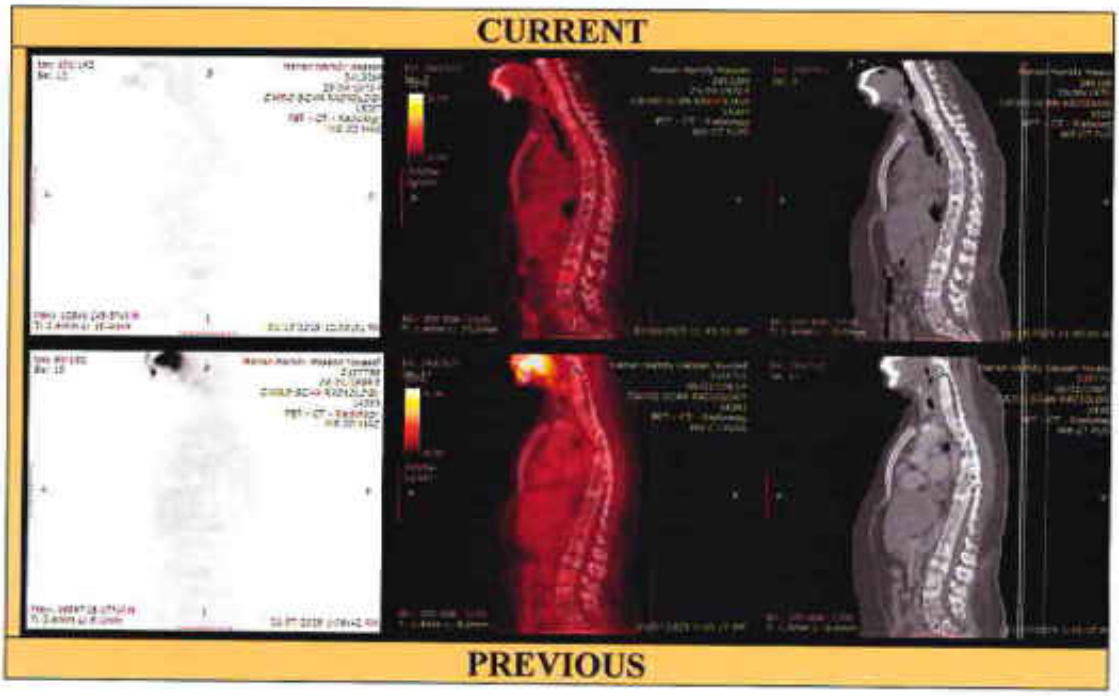
Imaging report

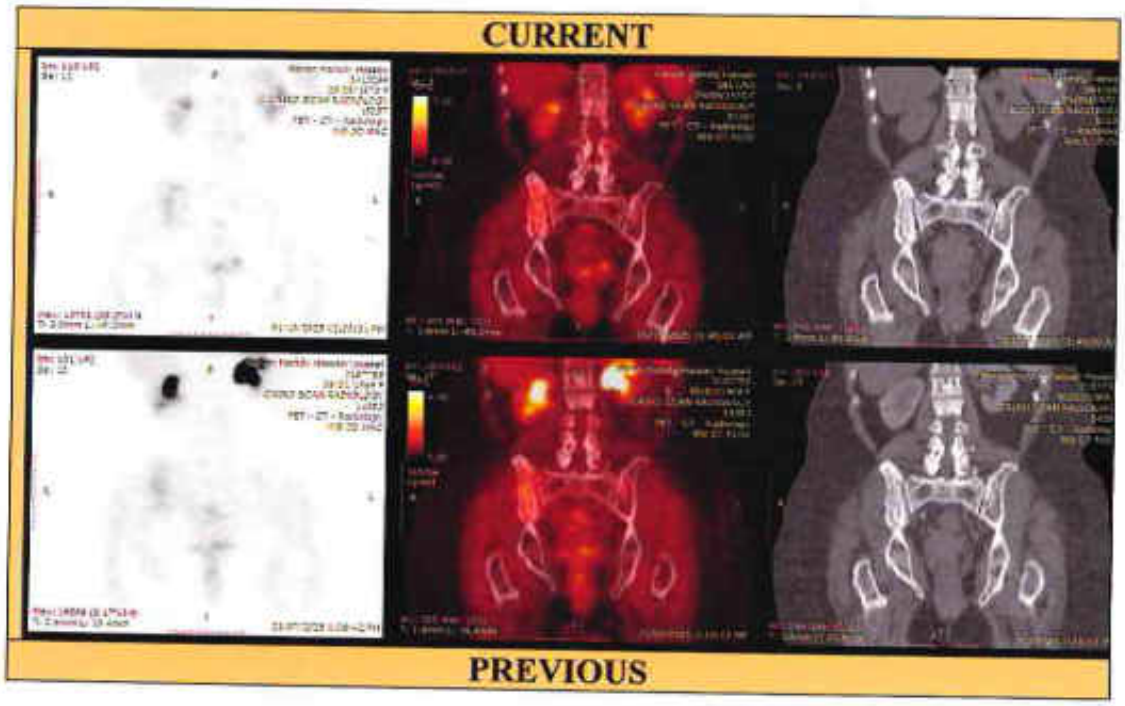
كايرو سكان
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CAIRO SCAN
Radiology & LABS

Pat. Name: Hanan Hamdy Hassan
Date: 01/10/2025

Pat. ID: 3413269

collapse of D6,D9 and L2 vertebrae , they show current with SUVmax3.5 compared to SUVmax3.1 in the prior study





Imaging report

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للأشعة والتحليل
CAIRO SCAN
Radiology & LABS

Pat. Name: Hanan Hamdy Hassan
Date: 01/10/2025

Pat. ID: 3413269

OPINION

Follow up PET CT study revealed:

- **Stationary course of the low grade metabolically active osseous deposits for correlation with serial tumor markers and for follow up**
- **No gross hypermetabolic lesions detected at the rest of the scanned body.**

Best Regards,
Prof. Dr. Maha Abd Elkareem
Professor of Nuclear Medicine
Kasr Al Ainy, Cairo University

For any inquiries regarding the findings in the report please contact us on
radreport@cairoscan.com.eg

Imaging report

كايرو سكان
للأشعة والتحليل
CAIRO SCAN
Radiology & LABS

Pat. Name: Hanan Hamdy Hassan Youssef
Date: 21/07/2025

Pat. ID: 3187758

^{18}F FDG WHOLE BODY FOLLOW UP PET/CT

Clinical Data:
Aim of the Study:
Previous PET/CT studies
Technique:

- Whole body 18F-FDG images from skull to the knees with the arms extending above the head were obtained approximately 1 hour after tracer injection.
- CT was acquired for the purpose of anatomic localization and attenuation correction
- Data was processed and displayed according to departmental protocol.

Review of PET. CT and fused images

Compared to last PET CT study dated 27-4-2025

BRAIN

- No evidence of intra-cerebral or extra-axial fresh blood density.
- Normal size and configuration of the ventricular system and subarachnoid basal cisterns as well as the cortical sulci on both sides.
- No definite cerebral areas of abnormal attenuation value or pathological enhancement.
- No shift of midline structures.
- Normal posterior fossa.
- Clear scanned para-nasal sinuses.

NECK:

- Normal CT appearance of valeculae, pyriform fossae and aryepigolottic folds.
- Patent nasopharyngeal and oro-pharyngeal air column.
- Bilateral deep cervical, subcentemetric benign looking lymph nodes, likely non specific.
- Normal CT appearance of the parotid and submandibular salivary glands.
- Normal appearance of the thyroid gland.
- Normal appearance of the carotid sheath and its contents

Imaging report

كايرو سكان للأشعة والتحليل
CAIRO SCAN
Radiology & LABS

Pat. Name: Hanan Hamdy Hassan Youssef
Date: 21/07/2025

Pat. ID: 3187758

CHEST:

- Status post operative , of the right breast showing parenchymal soft tissue distortion thickened skin and retracted nipple and deep surgical sutures with no pathological tracer uptake likely representing post operative sequelae. NO FDG avid recurrent neoplastic lesions detected
- Clear both lung fields with no evidence of metabolically active pulmonary nodules, masses, infiltrations or bronchiectatic changes.
- No pleural collection.
- Minimal pericardial effusion
- No metabolically active grossly enlarged hilar, mediastinal or axillary lymph nodes.
- Normal CT appearance of the heart, aorta and its great branches.
- Hiatus hernia noted

ABDOMEN AND PELVIS:

- Enlarged liver showing smooth regular outline with homogenous low parenchymal texture. No evidence of definite metabolically active enhanced focal hepatic masses or bile duct dilatation.
- Patent homogenously enhancing portal vein and its main branches.
- Normal CT appearance of the spleen, pancreas, suprarenal glands, kidneys, aorta, IVC and pelvic organs.
- No metabolically active grossly enlarged abdominal or pelvic lymph nodes.
- No ascites.

MUSCULO-SKELETAL

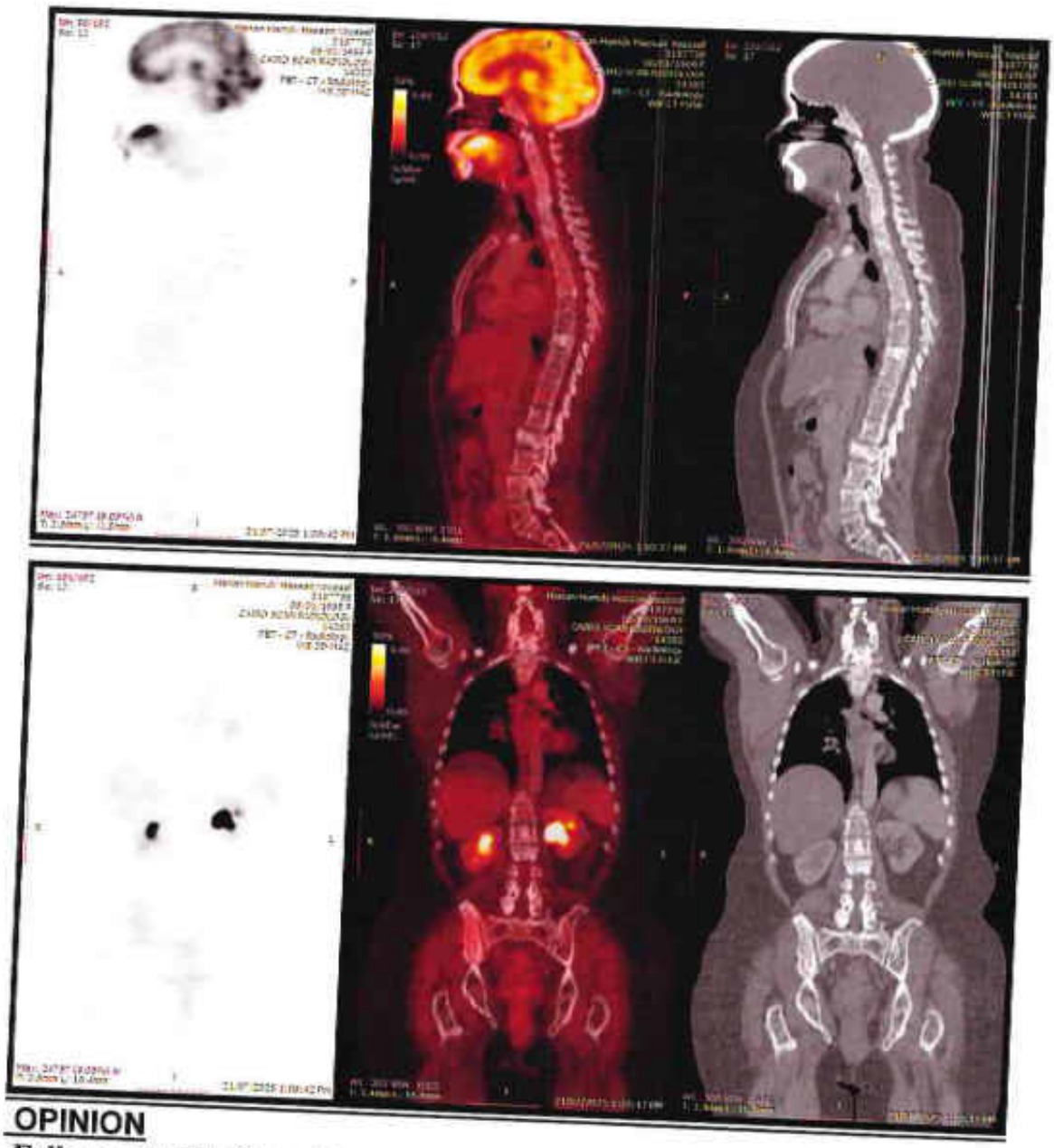
- No significant interval changes of the multiple mixed sclerotic and lytic skeletal lesions involving the axial skeleton ,both proximal humeri and femora including the Right sided osteolytic lesion centered upon the right iliac bone and right inferior pubic ramus with pathological fractures and reduced height/partial structural collapse of D6,D9 and L2 vertebrae with SUVmax3.1 compared to SUVmax3.5 in the prior study

Imaging report

كايرو سكان
للأشعة والتحليل
CAIRO SCAN
Radiology & LABS

Pat. Name: Hanan Hamdy Hassan Youssef
Date: 21/07/2025

Pat. ID: 3187758



OPINION

Follow up PET CT study revealed:

- **Stationary course of the low grade metabolically active osseous deposits for correlation with serial tumor markers and for follow up**
- **No gross hypermetabolic lesions detected at the rest of the scanned body.**

Best Regards,

Prof. Dr. Maha Abd Elkareem
Professor of Nuclear Medicine
Kasr Al Ainy, Cairo University

Dr. Hisham Adel,MD.

Imaging report

كايرو سكان
للأشعة والتحليل
CAIRO SCAN
Radiology & LABS

Name : Mrs. Hanan Hamdy Hassan Youssef

Date : 10/12/2024 Code : 3187758

3D FULL FIELD DIGITAL MAMMOGRAPHIC EXAMINATION OF BOTH BREASTS:

CLINICAL SUMMARY : conservative right breast cancer surgery.
IMPRESSION :
LEFT BREAST: (BIRADS 1)
Normal study

RIGHT BREAST: (BIRADS 2)
Post-operative changes

RECOMMENDATION :-
- For routine annual follow up with next examination due December 2025

DETAILED FINDINGS :

TECHNIQUE:

Craniocaudal & Mediolateral Oblique views
Tomosynthesis in 1mm slice thickness
High resolution ultrasound with color Doppler

Both breasts showed scattered fibroglandular tissue, (ACR b).

LEFT BREAST:

- No dominant mass lesion, spiculations or suspicious microcalcifications.
- This was confirmed by 3D Tomosynthesis 1mm section images
- Complementary ultrasound revealed no suspicious masses.
- No signs of malignancy detected.
- Normal skin/nipple/areolar complex.
- Preserved axillary lymph nodes.

Imaging report

كايرو سكان للأشعة والتحليل
CAIRO SCAN
Radiology & LABS

Name : Mrs. Hanan Hamdy Hassan Youssef

Date : 10/12/2024 Code : 3187758

RIGHT BREAST:

- Post-operative changes in the form of scarring at the UOQ, mild diffuse skin thickening..
- No suspicious mass lesion, spiculations or suspicious microcalcifications.
- This was confirmed by 3D Tomosynthesis 1mm section images and ultrasound.
- No signs of malignancy detected.
- No pathological axillary lymph nodes.

Thanks for your Reference,
Dr Hanaa Abo El Magd

For any inquiries regarding the findings in the report please contact us on
radreport@cairoscan.com.eg

Imaging report

كايرو سكان للأشعة والتحليل
CAIRO SCAN
Radiology & LABS

Name : Mrs. Hanan Hamdy Hassan Youssef

Date : 10/12/2024 Code : 3187758

Definition of Breast Imaging Reporting and Data System (BI-RADS):

N.B : Your Category is shaded below in gray

Category			
0	Needs further imaging study		
1	Negative (there is nothing to comment on)		
2	Benign Finding		
3	Probably Benign Finding	< 2% malignant risk	short term follow up is recommended
4	Suspicious Abnormality	2- 95% malignant risk	Biopsy should be considered
	4a	Small risk	
	4b	Moderate risk	
	4c	Substantial risk	
5	Highly suggestive of Malignancy	> 95% malignant risk	Appropriate Action should be taken
6	Known Biopsy	Proven Malignancy	

Imaging report

كايرو سكان
للأشعة والتحليل
CAIRO SCAN
Radiology & LABS

Name : Mrs. Hanan Hamdy Hassan Youssef
Date : 27/04/2025

Code : 3187758

Follow up F-18 Fluorodeoxyglucose (FDG) PET/CT of the whole body:

❖ **Clinical History:**

- female known with right breast cancer, underwent conservative surgery followed by CTH and RTH for assessment

❖ **Procedure:**

- Dose/Radiopharmaceutical: mCi F-18 Fluorodeoxyglucose (FDG) was administered via a peripheral upper limb vein.
- Blood glucose level: mg/dL.
- Imaging was performed on an integrated PET/CT scanner approximately 60 minutes after injection of the radiopharmaceutical.
- No IV contrast given.
- PET/CT images were reviewed in the transaxial, coronal and sagittal planes.
- The Standardised Uptake Value, maximum variant (SUV max) was calculated within regions of interest whenever needed

❖ **Findings**

Compared to previous PET CT dated 11/12/2024 in our PACS.

Head and Neck:-

- The brain appears normal on CT, with no abnormal focal lesion detected.
- No enlarged active cervical lymphadenopathy.
- Normal appearance of the oropharynx, larynx, thyroid and salivary glands.
- Normal appearance of the rest of the soft tissues of the neck.

Chest:

- Status post operative , of the right breast showing parenchymal soft tissue distortion thickened skin and

retracted nipple and deep surgical sutures , likely representing post operative sequelae. NO FDG avid recurrent neoplastic lesions detected.

- No pleural effusion
- Clear both lung fields with no FDG avid pulmonary nodules or other parenchymal abnormalities demonstrated.
- Minimal pericardial effusion noted
- No enlarged axillary, supra-clavicular, hilar or mediastinal lymph nodes could be detected.
- Normal appearance of the myocardium and great vessels.
- Slidung hiatus hernia.

Abdomen and Pelvis:

- Liver is of enlarged in size measuring about 17.1 cm along the MCL , showing homogenous texture and smooth outline.
- No hepatic focal lesions or biliary radicles dilatations could be noted on non contrast bases
- Normal CT appearance of the spleen, pancreas, adrenals, both kidneys, aorta and IVC
- Bulky RVF uterus
- No enlarged abdominal or pelvic lymph nodes.
- No ascites.

Muscio-skeletal:

- Moderate to marked metabolic REGRESSION of the old Multiple mixed blastic and lytic skeletal lesions involving the axial skeleton , both proximal humeri and femora including the Right sided osteolytic mass lesion centered upon the right iliac bone still the most active lesion with SUV max 3.5 (previously was 6.2).
- Bilateral sacroilitis
- Spondylodgenerative changes of the spine

Imaging report

كايرو سكان
للأشعة والتحاليل
CAIRO SCAN
Radiology & LABS

Name : Mrs. Hanan Hamdy Hassan Youssef
Date : 27/04/2025

Code : 3187758



OPINION :

Remarkable metabolic regression of the old wide-spread metastatic bone deposits as described.
NO other FDG avid metastases or loco-regional tumor recurrence.

Best Regards,

Dr. Ahmed Zaher , M.D.
Prof. Of Nuclear Medicine , NCI
Cairo University

PROF. MOHSEN KHALIL
Radiology Consultant

For any inquiries regarding the findings in the report please contact us on radreport@cairoscan.com.eg

Imaging report

كايرو سكان
CAIRO SCAN
Radiology & LABS

Pat. Name: Hanan Hamdy Hassan

Pat. ID: 3413269
Date: 04/02/2026

Whole Body F-18 FDG PET/CT EXAMINATION:

Clinical History:

- *Female patient with history of managed breast cancer.*
- ***PET/CT requested for assessment.***

Technique:

- *For the foremented patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCT was performed simultaneously and used for attenuation correction, anatomical localization and diagnosis.*



Comparison done with PET-CT dated 1-10-2025.

Report

Hepatic reference activity: 2.9

Regions of interest:

Status post right breast CBS; still no abnormal FDG avid lesions to suspect local tumor recurrence.



Dr.
Elia Anis Ishak
M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.
Shady Elia Anis
M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : حنان حمدى حسن يوسف
Analysis ID : 916563
Age : 55 Y : 1 M : 1 D
Gender : Female
Referred By Prof. Dr. : محمد حافظ
Clinical Diagnosis : History of resected breast cancer in 2020 and treated, presented with multiple bony lesions
Nature of Specimen : Block labelled as 915639 [Core biopsy]
Receiving Date : 2024-12-19
Patient ID : 683796

IMMUNOHISTOCHEMISTRY REPORT

Technique :

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain
The sections were treated against:
GATA 3.

Results :

- Tumor cells are POSITIVE for GATA 3.

Conclusion :

Block, FINDINGS ARE COMPATIBLE WITH METASTATIC CARCINOMA OF BREAST DUCT ORIGIN.
- REQUESTED TESTS IN SEPARATE REPORT.

Prof. Dr. Shady Elia Anis ***Prof. Dr. Elia Anis Ishak***

مستشفى دار الفؤاد
مدينة نصر
القاهرة

This report is electronically signed , any abrasion or modification would render it void .

عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس ، ت: ٠٢٢٢٢٢٢٢٢

Dr.
Elia Anis Ishak
M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.
Shady Elia Anis
M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
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Patient Name : حنان حمدى حسن يوسف
Analysis ID : 916561
Age : 55 Y : 1 M : 1 D
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Referred By Prof. Dr. : مجد حافظ
Clinical Diagnosis : History of resected breast cancer in 2020 and treated, presented with multiple bony lesions
Nature of Specimen : Block labelled as 915639 [Core biopsy]
Receiving Date : 2024-12-19
Patient ID : 683796

IMMUNOHISTOCHEMISTRY REPORT

Technique :

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain.
The sections were treated against:
- Estrogen receptors -Progesterone receptors - Her 2 - Ki67.

Results :

90% of tumor cells show strong POSITIVE Staining for Estrogen receptors.
Sections treated for Progesterone receptors revealed NEGATIVE nuclear staining.
Sections treated for HER2 revealed NEGATIVE CELL MEMBRANE STAINING with membranous staining in 5% of tumor cells.
25% of Tumour cells are POSITIVE for Ki67.

Conclusion :

Block, TUMOUR CELLS ARE;
POSITIVE FOR ESTROGEN RECEPTORS (SCORE 8 / 8).
NEGATIVE FOR PROGESTERONE RECEPTORS (SCORE 0 / 8).
HER 2 NEGATIVE, SCORE 0.
THE PROLIFERATION INDEX OF Ki67 IS 25% OF THE TUMOR CELLS.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak

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Dar Al Fouad
Hospital

Patient Name: HANAN HAMDY HASSAN
Patient ID: 20153474
Study Date: 17/12/2024 10:47:46 ã
Age: 55 Years

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH CORONAL RECONSTRUCTION: -

Findings:

- Bilateral mild pleural effusion with bilateral lower lung lobes thick fibro-ateclacitc bands
- No hilar or mediastinal lymph nodes.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. Mild pericardial effusion detected.
- Normal non-enhanced appearance of the thoracic aorta and the great vessels.
- Normal appearance of the chest wall.
- Normal appearance of the dorsal vertebrae.
- Upper abdominal cuts are unremarkable.

OPINION:

- Bilateral mild pleural effusion with bilateral lower lung lobes thick fibro-ateclacitc bands
- Mild pericardial effusion

Ahmed seif
Date & Time: 18/12/2024 01:15:40 Õ

Dr. Ahmed Seif M.D

Dar Al Fouad
Hospital

Patient Name: HANAN HAMDY HASSAN
Patient ID: 20153474
Study Date: 18/12/2024 01:57:53 Ô
Age: 55 Years

CT ANGIOGRAPHY OF THE PULMONARY VESSELS:-

Technique:

- **100 cc of non ionic contrast material was injection via a superficial upper limb venous cannula, followed by spiral acquisition of the chest in pulmonary and arterial phases.**
- **Multiplaner reformation with MIP and volume rendering reconstruction was done.**

Findings:

- Patent pulmonary artery and its main branches down to the 3rd order branches with no filling defects or aneurysm formation.
- Normal CT appearance of the thoracic aorta and the great vessels.

OPINION:

- **No evidence of pulmonary embolism.**

Ahmed seif

Date & Time: 18/12/2024 08:44:48 Ô

Dr. Ahmed Seif M.D

مستشفى دار الفؤاد
Dar Al Fouad Hospital

Patient Name : Mrs / / Hanan Hamdy Hassan Youssef
Age : 55 Y
Sex : Female
Ref.Doctor :
Reg.Branch : NASR CITY

Patient No. : 0020153474
Acc.No. : 12496514
Sample Date : 06/12/2024 01:39 PM
Report Date : 06/12/2024 02:17 PM
Printed Date : 06/12/2024 03:46 PM

(CBC) Complete Blood Picture

Test	Result	Unit	Ref. Range
T.L.C	14.13	x10³/uL	4 - 10
Haemoglobin	10.50	g/dL	12 - 15
Hematocrit	31.4	%	36 - 46
Red cell count	3.57	x10⁶ /uL	3.8 - 4.8
MCV	88.0	fL	80 - 100
MCH	29.4	pg	27 - 32
MCHC	33.4	g/dL	31.5 - 34.5
RDW	12.1	%	11.5 - 14
Platelet Count	181	x10³/uL	150 - 410
MPV	11.0	fL	7.5 - 8.3
NRBCs	0.0	%	
NRBCs absloute	0.00		

Differential leucocyte Count

	Relative Result	Unit	Ref. Range	Absolute Count Result	Unit	Ref. Range
Basophils :	0.3	%	0 - 1	0.04	x10³/uL	Up To 0.1
Eosinophils :	0.4	%	1 - 6	0.06	x10³/uL	0.02 - 0.5
Stab :	4.0	%		0.56	x10³/uL	
Neutrophils :	79.8	%	40 - 80	11.84	x10³/uL	2 - 7
Lymphocytes :	11.7	%	20 - 40	1.65	x10³/uL	1 - 3.5
Monocytes	3.8	%	2 - 10	0.54	x10³/uL	0.2 - 1

Comments :
Follow up CBC.

Lab Manager :
Dr. Samah Mohamed

Reviewed By:

Mariam Abdelmesseh

مستشفى دار الفؤاد
Dar Al Fouad Hospital

Patient Name : . / HANAN HAMDY HASSAN YOUSEF
Age : 55 Y
Sex : Female
Ref.Doctor :
Reg.Branch : MainBranch

Patient No. : 0011012647
Acc.No. : 224121985
Sample Date : 21/12/2024 01:21 PM
Report Date : 22/12/2024 01:09 PM
Printed Date : 02/01/2025 04:27 PM

Chemistry Unit

Test	Result	Unit	Ref. Range
ILH (Ictric-Lipemia-Hemolysis)	0		0
Liver Function			
Protein, Total	5.9	g/dl	6.6 - 8.3
Bilirubin (Direct)	0.08	mg/dl	< 0.2
Bilirubin (Total)	0.63	mg/dL	Less Than 1.2
AST (S.G.O.T)	44	U/L	10 - 35
ALT (S.G.P.T)	74	IU/L	10 - 35
Albumin, Serum	3.5	g/dl	3.5 - 5.2
Alkaline Phosphatase	312	U/L	35 - 104
A/G Ratio	1.46		1 - 2

Reviewed By:

Dr. Bassem Eldawwy

Lab Manager :
Dr. ShereenHamdy

مستشفى دار الفؤاد
Dar Al Fouad Hospital

Patient Name	: . / HANAN HAMDY HASSAN YOUSEF	**Patient No.**	: 0011012647
Age	: 55 Y	**Acc.No.**	: 224122401
Sex	: Female	**Sample Date**	: 22/12/2024 02:49 PM
Ref.Doctor	:	**Report Date**	: 22/12/2024 05:07 PM
Reg.Branch	: MainBranch	**Printed Date**	: 02/01/2025 04:25 PM

Chemistry Unit

Test	Result	Unit	Ref. Range
Creatine Kinase (CK)Total	**227**	U/L	**Less Than 170**
Factors that may affect test results include cardiac catheterization, intramuscular injections,trauma to muscles, recent surgery, and heavy exercise.			
Creatine Kinase MB (CK-MB)	**mass 1.14**	ng/ml	**0.5 - 5**
Troponin T highly Sensitive	**22.2**	pg/ml	**< 5 : low risk** **5 - 52 : intermediate risk** **> = 52 : high risk**

Reviewed By:

Lab Manager :
Dr. ShereenHamdy

Dr. Bassem Eldawwy
Ghada Helmy Elashram

Dar Al Fouad Hospital
In Collaboration with CLEVELAND CLINIC INTERNATIONAL

Patient Name : HANAN HAMDY HASSAN	Date : 20/12/2024
MR# : 11012647	DLP===mGycm

DOUBLE CONTRAST BARIUM SWALLOW AND MEAL EXAMINATION REVEALED:

- Free flow of the contrast through the esophagus into the stomach with no evidence of obstruction, stricture, filling defects or diverticulae.
- Normal mucosal pattern with no detectable ulceration.
- Normal duodenal cap and duodenal curve.
- No evidence of reflux or hiatus hernia.
- Normal appearance of the gastro-esophageal junction.

Opinion:

- ***Normal double contrast barium swallow and meal study.***

Dr. Yousef Ahmed , MD

Visit Number	: 3243489	**Patient Number**	: 15-11721
Name	: الاستاذة/ حنان حمدى حسن يوسف	**Age / Sex**	: 55 Y Female.
Registration Date	: 01-12-2024 07:28 PM	**Result Date**	: 02-12-2024 04:59 PM
Referred By	: بدون روشته		

Haematology

COMPLETE BLOOD PICTURE

Comments :

BLOOD PICTURE SHOWS MILD MICROCYTIC HYPOCHROMIC ANAEMIA.
TLC IS WITHIN NORMAL LIMITS.
PLATELETS ARE NORMAL.

Head of unit :
Dr. Prof.Dr.ahmedellaweel[CMO]

Reviewed by
Dr. abdelrahman magdy abdelmonsef

Printed By: Shahinaz Hosny Ibrahiem

Printed Date: 04/12/2024 11:04 AM

المركز الرئيسي : ٤ شارع شهاب، المهندسين
Central Lab : 4 Shehab Street,Mohandessin
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Hotline الخط الساخن 19962

RIQAS ISO TÜV AUSTRIA

Visit Number	3489	**Patient Number**	: 15-11721
Name	: الاستاذة/ حنان حمدى حسن يوسف	**Age / Sex**	: 55 Y Female
Registration Date	: 01-12-2024 07:28 PM	**Result Date**	: 02-12-2024 04:59 PM
Referred By	: بدون روشته		

Haematology
COMPLETE BLOOD PICTURE

				Ref. Range
Red cell count	4.11		x10^6/cmm	3.8 - 4.8
Hemoglobin	10.4	L	g/dl	12 - 15
Haematocrit HCT	32.9	L	%	36 - 45
MCV	80.0	L	fl	84 - 101
MCH	25.3	L	pg	27 - 32
MCHC	31.6		%	31.5 - 34.5
R.D.W.	12.9		%	11.6 - 14
Platelet count	160		10^3/cmm	150 - 450
White cell count.	8.67		10^3/cmm	4 - 11

Interpretive Criteria :

Differential leucocyte Count

	Relative			Absolute	
	Result %		Normal	Result /cmm	Normal
Basophils	0	%	0 - 2	0	0 - 100
Eosinophils	1	%	1 - 6	87	20 - 500
Monocytes	7	%	1 - 10	607	200 - 1,000
Lymphocytes	20	%	20 - 45	1734	1,000 - 4,000
Neutrophils	72	%	40 - 80	6242	2,000 - 7,000
Seg.	70	%	0 - 80	6069	
Bands	2	%	0 - 6	173	

Head of unit :
Dr. Prof.Dr.ahmedeltaweel[CMO]

Reviewed by
Dr. abdelrahman magdy abdelmonsef

Printed By: Shahinaz Hosny Ibrahiem

Printed Date: 04/12/2024 11:04 AM

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Visit Number	: 15243489	**Patient Number**	: 15-11721
Name	: الاستاذة/ حنان حمدى حسن يوسف	**Age / Sex**	: 55 Y Female
Registration Date	: 01-12-2024 07:28 PM	**Result Date**	: 02-12-2024 03:34 PM
Referred By	: بدون روشته		

Chemistry

Test	Result	Unit	Ref. Range
SGOT (AST)	30	U/L	0 - 35
SGPT (ALT)	28	U/L	0 - 35
Blood Urea	125.0 H	mg/dl	13 - 50
Creatinine (Serum)	3.57 H	mg/dl	0.51 - 1.09
Uric Acid (Serum)	7.20 H	mg/dl	2.4 - 5.7
Hb A1-C	5.6	%	4 - 6.5 Normal : = 4.0 - 5.6 Pre-diabetic: 5.7 - 6.4 Diabetic : = / > 6.5

Head of unit :
Dr. Prof.Dr.ahmedeltaweel[CMO]

Reviewed by
Sahar Ali

Printed by: Shahinaz Hosny Ibrahiem

Printed Date: 04/12/2024 11:04 AM

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جمهورية مصر العربية
بطاقة تحقيق الشخصية
حنان
حمدى حسن يوسف
١٤٦ ش فيصل - برج خالد
الهرم - الجيزه
٢٦٩ ٠١ ٠٦ ٢٣ ٠٠١ ٠٣
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